Fax cover sheet

Sunridge Community Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Gabriel Montoya, MSW
Phone: 555-0186
Date: 10/21/2026 10:20
Pages (including cover): 10
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

61F lower GI bleed s/p transfusion, Hgb stable.
Medically ready for d/c. Requesting recuperative care.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Sunridge Community Hospital (fictional)
Dept/Floor: 3 North Medical
SW/CM/RN: Gabriel Montoya, MSW
Phone/Pager #: 555-0186
Email: gmontoya@sunridgecommunity.example
Nursing Station #: 555-0187
Authorized by: Dr. A. Brandt
Phone #:

Patient Information

Patient Name: Ruth Doe MRN: SR-2207461 Date of Birth (dd/mm/yyy): 11/02/1964
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: White
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Lower GI bleed, acute blood loss anemia
Admit Date: 10/17/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/17/2026 Results: CXR clear

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* X No	Active Infection	Yes* X No
*Explain		*Explain	
Self Administer All Meds	X Yes No*	Ambulatory	X Yes No
Continent of Bowel & Bladder	X Yes No	Assistive Device	Yes* X No
Colostomy/Ileostomy	Yes X No	*Explain	
Foley Catheter	Yes X No	Hospice	Yes* X No
Diabetes	Yes X No	*Explain	
Insulin	Yes X No	Car	Yes X No
Diabetes related Oral Meds	Yes X No	Service Animal	Yes X No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain none

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Ruth Doe

DOB: 11/02/1964

Member ID: OCH-73318504

Referring Provider

Facility: Sunridge Community Hospital (fictional)

Clinical

Primary Diagnosis: Diverticulosis of large intestine with bleeding

ICD-10: K57.31

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 10/21/2026

Requested Days: 14

Secondary Dx: Acute posthemorrhagic anemia (D62)

Plan Use Only

Status: Pending review

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Patient face sheet

Sunridge Community Hospital (fictional) - PATIENT FACE SHEET

Patient: Ruth Doe
MRN: SR-2207461
Account #: 99207461
DOB: 11/02/1964
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/17/2026 03:40
Admit Type: Inpatient
Unit/Bed: 3N-318
Attending: Anika Brandt, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date of Service: 10/17/2026

Attending: Anika Brandt, MD (Hospital Medicine)

Chief Complaint

Bright red blood per rectum x 1 day, lightheaded

HPI

61 y/o F experiencing homelessness (staying in a shelter bed at Example Shelter x 3 months) presents with 3 episodes of painless bright red blood per rectum and lightheadedness. Denies hematemesis or melena. Denies NSAID use. In ED: HR 112, BP 98/60, Hgb 6.8.

PMH

- Diverticulosis (per pt, colonoscopy years ago)
- Hypothyroidism

PSH

- None

Social History

Homeless x 3 months, shelter. Denies tobacco, alcohol or drug use.

Allergies

Penicillin (rash)

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History and Physical - page 2

Physical Exam

VS: T 98.2F, HR 112, BP 98/60, RR 18, SpO2 99% RA
Gen: pale, tired, AOx4
CV: tachycardic, regular
Abd: soft, non-tender, non-distended
Rectal: maroon and bright red blood, no masses
Psych: calm, cooperative

Assessment / Plan

61F with acute lower GI bleed, likely diverticular, with acute blood loss anemia.
1. LGIB - 2 large bore IVs, transfuse 2 units PRBC, serial Hgb q6h, GI consult for colonoscopy
2. Hold DVT chemoprophylaxis; SCDs
3. MRSA nares PCR, COVID per protocol
4. Dispo - CM consult; homeless

Isolation: None - standard precautions
Code status: Full
Electronically signed: Anika Brandt, MD 10/17/2026 05:15

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Gastroenterology consult and colonoscopy

GASTROENTEROLOGY CONSULT / PROCEDURE NOTE

Date: 10/18/2026 13:30
Consultant: Oren Takahashi, MD (Gastroenterology)
Procedure: Colonoscopy after bowel prep
Findings: Extensive sigmoid and descending colon diverticulosis. No active bleeding. No stigmata of recent hemorrhage requiring intervention. No masses, no colitis.
Upper endoscopy not indicated (no hematemesis, BUN not elevated).
Impression: Diverticular bleed, self-limited. No ulcer disease. No varices.
Recommendations: High fiber diet. Avoid NSAIDs. Outpatient GI follow-up in 4-6 weeks. OK for discharge from GI standpoint if Hgb stable.
Signed: Oren Takahashi, MD

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 10/21/2026 08:00

S: No further bleeding since 10/17 PM. Lightheadedness resolved. Tolerating diet.

O: T 98.1F HR 82 BP 122/74 RR 16 SpO2 99% RA

Hgb 9.1 (10/20) and 9.0 (10/21), stable on two checks; s/p 2 units PRBC 10/17.

No bowel movements with blood x 72h.

A/P:

- Diverticular LGIB, resolved. Hgb stable. GI follow-up 4-6 weeks.
- Acute blood loss anemia - start oral iron.
- Hypothyroidism - continue home levothyroxine.
- Medically stable for d/c to recuperative care today ~14:00.

Signed: Anika Brandt, MD

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Case management and social work note

CASE MANAGEMENT / SOCIAL WORK NOTE

Date: 10/21/2026 09:00

Pt AOx4, pleasant and cooperative. No behavioral concerns per nursing.

Psychosocial screen: denies depression. Denies SI/HI. Denies auditory or visual hallucinations. No psychiatric history. No history of conservatorship or FSP.

Substance use: denies.

Not a registered sex offender per pt.

Pt agreeable to recuperative care.

GI follow-up: to be scheduled by GI clinic after d/c.

Signed: Gabriel Montoya, MSW

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Vitals and labs

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/20 08:00	98.4	86	120/72	16	99%	RA
10/20 16:00	98.2	84	118/74	16	98%	RA
10/21 00:00	98.0	78	116/70	14	99%	RA
10/21 04:00	98.1	80	118/72	16	99%	RA
10/21 08:00	98.1	82	122/74	16	99%	RA

LABS

10/17 02:50 | Hgb 6.8 L | Plt 242 | BUN 14 | Cr 0.8 | INR 1.0
10/18 05:00 | Hgb 8.6 L (post 2 units PRBC)
10/20 05:00 | Hgb 9.1 L | Cr 0.7
10/21 05:00 | Hgb 9.0 L | TSH 2.1

MICROBIOLOGY

10/17 MRSA nares PCR: Negative
10/21 06:00 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/17 CXR: clear lungs

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